✅ THE MOST COMMON QUESTIONS LILY ASKS (CL VIVA EDITION)

Below is the pattern she repeats in literally every scenario. If you can answer these, you're safe 90 percent of the time.

🔵 1. HISTORY TAKING — WHAT TO ASK + WHY

She *always* pushes "WHY?" so these are the safe, exam-friendly reasons.

History questions

- Previous CL wear?
- Wearing schedule (hours/day, days/week)
- Motivation & lifestyle
- Occupation & environment + VDU
- Hobbies
- General health
- Medications
- Allergies/atopy
- Ocular history: infections, injuries, surgeries
- Contact with water (swimming, showering)

Her "but WHY?" follow-ups

- **Previous CL wear** → tells you brand, modality, comfort issues, past complications.
- **Occupation** → dusty environments = DD preferred; office work = dryness considerations.
- **Allergies** → active allergic conjunctivitis is a contraindication; atopy increases CLPC risk.
- **Medications** → antihistamines, SSRIs, antipsychotics, beta blockers = dryness.
- **Diabetes** → slower epithelial healing + reduced corneal sensitivity → closer monitoring.
- **Thyroid/Graves** → exposure keratopathy → dryness → manage expectations.
- **Injuries/surgeries** → scarring, irregularity → affects lens choice.

🔵 2. CONVERTING PRESCRIPTIONS

She LOVES this.

Her go-to questions

- "What does this Rx convert to for CLs?"
- "Do you correct BOTH sphere and cyl?"
- "Do you get a calculator?"
- "Do you compensate for nasal rotation?"

Your safe answers

- Yes, vertex both sphere and cyl
- No calculators.
- Only compensate for nasal torque if lens axis increments force you to choose between 170 or 180 etc.
    - Nasal rotation is usually 5–10 degrees.

🔵 3. CHOOSING A LENS

She will always ask:

- “What lens would you choose?”
- “Why that lens?”
- “Why SiHy?”
- “What is modulus?”
- “What is Dk/t and why is it important?”
- “What are signs of hypoxia?”

Safe bullet responses

- **SiHy**: higher Dk/t not dependent on water; better for thicker high-power torics.
- **Modulus**
    - High modulus = easier handling, but more mechanical complications
    - Low modulus = better comfort, less SEAL/CLPC.
- **Dk vs Dk/t**
    - Dk = permeability of material.
    - Dk/t = oxygen transmissibility through that specific lens thickness.
- **Hypoxia signs**:
    - Microcysts
    - Vacuoles
    - Stromal oedema, striae, folds (5 percent vs 8 percent thresholds)
    - Endothelial blebs
    - Polymegathism/pleomorphism
    - Neovascularisation

🔵 4. SWIMMING QUESTIONS

Always, ALWAYS comes up.

Her usual sequence

1. “Patient wants to swim in their CLs. What do you tell them?”
2. “Why can’t they wear MONTHLYS in water?”
3. “Can you clean off acanthamoeba?”
4. “Why not heat disinfection?”
5. “What DD would you give instead?”
6. “Do you spherical-equivalent the cyl they can’t get in DDs?”

Safe answers

- No swimming in monthlies ⟶ acanthamoeba cysts survive disinfectants
- DD only; discard immediately
- Wear tight goggles
- Wait **15–20 minutes** before removing after swimming.
- MPS kills trophozoites only; NOT cysts
- Heat disinfection denatures lens proteins, damages lenses, not used anymore.

🔵 5. RGP QUESTIONS

She will hit these even if your case is SCL.

- “Would you consider RGPs?”
- “What more information do you need?”
- “What topo features are useful?”
- “Which RGP design would you pick?”
- “Difference between SPE and CPE?”
- “How do toric RGPs stabilise?”

Safe answers

- Need topo:
    - Corneal toricity
    - Eccentricity
    - K values
    - HVID (for diameter)
- SPE vs CPE
    - **SPE**: front surface + back toric ⟶ masks induced cyl ⟶ rotation independent.
    - **CPE**: corrects both refractive and induced astig ⟶ needs prism to stabilise.
- Stabilisation:
    - Prism ballast (1.5–2Δ BD)
    - Mandell-Moore fitting based on corneal toricity.

🔵 6. ORTHOK QUESTIONS

Comes up a lot with high cyl scripts.

She’ll ask

- “Is this patient suitable for orthok?”
- “What would you check?”
- “What do you WANT to see on topo?”

Safe answers

Ideal orthok candidate

- <1.50 DC corneal toricity
- WTR astig

- Not limbal-to-limbal astig
- Flat K <40D
- Normal e-value (not steep)
- Small pupil
- No tight lids

🔵 7. SOLUTIONS + DISINFECTION QUESTIONS

She'll ask

- "What solution would you recommend?"
- "What are the active ingredients?"
- "Does it work for acanthamoeba?"
- "What about hydrogen peroxide?"

Safe answers

Optifree PureMoist

- Polyquad: antimicrobial
- Aldox: kills acanthamoeba *trophozoite* only
- Ineffective on cysts

Hydrogen peroxide

- Great for bacteria/fungi
- Also poor for cysts unless combined with rubbing
- No solution is fully effective against cysts

🔵 8. RANDOM BUT GUARANTEED QUESTIONS

She has asked these literally every day:

- "Why do you need to vertex BOTH sphere and cyl?"
- "Do toric DDs come in your cyl?"
- "Why not XR for this eye?"
- "Why would you NOT pick RGPs?"
- "Why don't we heat disinfect anymore?"
- "What is the complication of high plus in RGP?" – Low riding lens
- "How do you fix a low rider?"
    - Increase TD
    - Steepen BOZR